M DMH Medication-Assisted Treatment (MAT)

Medication-assisted treatment (MAT) is the use of medications, in combination with counseling and behavioral therapies to provide a “whole-patient” approach to the treatment of substance use disorders. These services are provided to recipients on the basis of medical necessity.

M.1. Enrollment

For the provision of Substance Abuse Rehabilitative Services an entity:

- Must be an organization that is currently certified by the Alabama Department of Mental Health (DMH) to provide alcohol and other drug treatment services under the provisions of Chapter 580 of the Alabama Administrative Code; and
- Must submit an application to and receive approval by DMH to provide Substance Abuse Rehabilitative Services under the Medicaid Rehabilitative Option program.
- A provider must demonstrate the capacity to provide services off-site in a manner that assures the recipient's right to privacy and confidentiality and must demonstrate reasonable access to services as evidenced by service location(s), hours of operation, and coordination of services with other community resources.
- A provider must ensure that Medicaid recipients receive quality services in a coordinated manner and have reasonable access to an adequate array of services delivered in a flexible manner to best meet their needs and must have demonstrated the capacity to provide these services.

M.2. Eligible Providers

The eligible practitioners, the qualifications required and services furnished for those who may provide MAT services under the Rehabilitation Option Program are as follows:

- A **physician** licensed under Alabama law to practice medicine or osteopathy; Medical Assessment and Treatment; Intake Evaluation; Individual Counseling; Family Counseling; Group Counseling; Treatment Plan Review; Mental Health and Substance Use Disorders Assessment Update; Psychoeducational Services; Opioid Use Disorder Treatment.
- A **physician assistant** licensed under Alabama law and practicing within the guidelines as outlined by the Alabama Board of Medical Examiners; Medical Assessment and Treatment; Intake Evaluation; Individual Counseling; Family Counseling; Group Counseling;

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Treatment Plan Review; Mental Health and Substance Use Disorders Assessment Update; Psychoeducational Services; Opioid Use Disorder Treatment.

- A **Certified Registered Nurse Practitioner** (CRNP) licensed under Alabama law practicing within the scope as defined by the Joint Committee of the Alabama Board of Nursing and the Alabama Board of Medical Examiners for Advanced Practice Nurses; Medical Assessment and Treatment; Treatment Plan Review; Opioid Use Disorder Treatment.
- A **Certified Registered Psychiatric Nurse Practitioner** (CRNP) licensed under Alabama law practicing within the scope as defined by the Joint Committee of the Alabama Board of Nursing and the Alabama Board of Medical Examiners for Advanced Practice Nurses; Intake Evaluation; Individual Counseling; Family Counseling; Group Counseling; Treatment Plan Review; Mental Health and Substance Use Disorders Assessment Update; Psychoeducational Services; Opioid Use Disorder Treatment.
- A **psychologist** licensed under Alabama law; Intake Evaluation; Individual Counseling; Family Counseling; Group Counseling; Treatment Plan Review; Mental Health and Substance Use Disorders Assessment Update; Psychoeducational Services.
- A **professional counselor** licensed under Alabama law; Intake Evaluation; Individual Counseling; Family Counseling; Group Counseling; Treatment Plan Review; Mental Health and Substance Use Disorders Assessment Update; Psychoeducational Services.
- A **certified social worker** licensed under Alabama law; Intake Evaluation; Individual Counseling; Family Counseling; Group Counseling; Treatment Plan Review; Mental Health and Substance Use Disorders Assessment Update; Psychoeducational Services.
- A **marriage and family** therapist licensed under Alabama law; Intake Evaluation; Individual Counseling; Family Counseling; Group Counseling; Treatment Plan Review; Mental Health and Substance Use Disorders Assessment Update; Psychoeducational Services.
- A **registered nurse** licensed under Alabama law who has completed a master's degree in psychiatric nursing; Intake Evaluation; Opioid Use Disorder Treatment.
- A **registered nurse** licensed under Alabama state law; Psychoeducational Services; Opioid Use Disorder Treatment
- A **practical nurse** licensed under Alabama state law; Opioid Use Disorder Treatment.
- **Qualified Substance Abuse Professional (QSAP) I**: A Qualified Substance Abuse Professional I shall consist of: (i) An individual licensed in the State of Alabama as a: (I) Professional Counselor, Graduate Level Social Worker, Psychiatric Clinical Nurse Specialist, Psychiatric Nurse Practitioner, Marriage and Family Therapist, Clinical Psychologist, Physician's Assistant, Physician; or (ii) An individual who: (I) Has a master's Degree or above from a nationally or regionally accredited university or college in psychology, social work, counseling, psychiatric nursing, and * (II) Has successfully

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completed a clinical practicum or has six month's post master's clinical experience; and * (III) Holds a substance abuse counselor certification credential from the Alabama Association of Addiction Counselors, National Association of Alcoholism and Drug Abuse Counselors, Alabama Alcohol and Drug Abuse Association, or International Certification and Reciprocity Consortium/Alcohol and Other Drug Abuse, Inc. which shall be obtained within thirty (30) months of date of hire. Intake Evaluation.

- **QSAP II** shall consist of: (i) An individual who: (I) Has a Bachelor's Degree from a nationally or regionally accredited university or college in psychology, social work, community-rehabilitation, pastoral counseling, family therapy, or other behavioral health area that requires equivalent clinical course work, and (II) Is licensed in the State of Alabama as a Bachelor Level Social Worker; or (III) Has a Bachelor's Degree from a nationally or regionally accredited college or university in psychology, social work, community-rehabilitation, pastoral counseling, family therapy, or other behavioral health area that requires equivalent clinical course work, and (IV) Holds a substance abuse counselor certification credential from the Alabama Association of Addiction Counselors, National Association of Alcoholism and Drug Abuse Counselors, Alabama Alcohol and Drug Abuse Association, or International Certification and Reciprocity Consortium. Individual Counseling; Family Counseling; Group Counseling.
- **QSAP III** shall consist of: (i) An individual who: (I) Has a Bachelor's Degree from a nationally or regionally accredited university or college in psychology, social work, community-rehabilitation, pastoral counseling, family therapy, or other behavioral health area that requires equivalent clinical course work, and (II) Participates in ongoing supervision by a certified or licensed QSAP I for a minimum of one (1) hour individual per week until attainment of a substance abuse counselor certification credential from the Alabama Association of Addiction Counselors, National Association of Alcoholism and Drug Abuse Counselors, or Alabama Alcohol and Drug Abuse Association, or International Certification and Reciprocity Consortium/Alcohol and Other Drug Abuse, Inc. which shall be obtained within thirty (30) months of hire; Psychoeducational Services.
- Certified Recovery Support Specialist (CRSS) must meet the following minimum qualifications: (i) Certified by ADMH as a Certified Recovery Support Specialist (CRSS) within six (6) months of date of hire, (ii) and has 2 years verified lived experience and (iii) Concurrent participation in clinical supervision by a licensed or certified QSAP I; Peer Support Services

M.3. Covered Services

Intake Evaluation (90791-HF)

Initial clinical evaluation of the recipient's request for assistance. Substance abuse recipients undergo standardized psychosocial assessment. The intake evaluation presents psychological and social functioning, recipient's reported physical and medical condition, the need for additional evaluation and/or

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treatment, and the recipient's fitness for Medication Assisted Treatment (MAT) services.

Billing Unit:	Episode
Maximum Units:	Unlimited
Billing Restrictions:	May not be billed in combination with Treatment Plan Review (H0032)

Medical Assessment and Treatment (H0004-HF)

Face-to-face contact with a recipient during which a qualified practitioner provides psychotherapy and/or medical management services. Services may include physical examinations, evaluation of co-morbid medical conditions, development or management of medication regimens, the provision of insight oriented, behavior modifying, supportive, or interactive psychotherapeutic services, or the provision of educational services related to management of a substance use disorder.

Billing Unit:	15 minutes
Maximum Units:	6 per day, 52 per year
Billing Restrictions:	May not be billed in combination with Partial Hospitalization (H0035), Outpatient Detoxification – Ambulatory Detoxification With Extended On-Site Monitoring (II-D) (H0013), Outpatient Detoxification – Ambulatory Detoxification Without Extended On-Site Monitoring (I-D) (H0014).

Individual Counseling (90832-HF/90834-HF/90837-HF)

The utilization of professional skills by a qualified practitioner to assist a recipient in face-to-face, one-to-one psychotherapeutic encounter in achieving specific objectives of treatment or care for a mental health and/or a substance use disorder. Services are generally directed toward alleviating maladaptive functioning and emotional disturbances relative to a mental health and/or substance use disorder, and restoration of the individual to a level of functioning capable of supporting and sustaining recovery. Individual Counseling may consist of insight oriented, behavior modifying, supportive, or interactive psychotherapeutic services.

BILLING UNIT:	1 UNIT
Maximum Unit:	1 per day, 52 per year
Billing Restrictions:	May not be billed in combination with Partial Hospitalization (H0035), Opioid Use Disorder Treatment (H0020), Outpatient Detoxification – Ambulatory Detoxification With Extended On-Site

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Monitoring (II-D) (H0013), Outpatient Detoxification – Ambulatory Detoxification Without Extended On-Site Monitoring (I-D) (H0014).

Additional Information: Billing = 1 of the following codes:
- Code 90832 = therapy given for 16 to 37 minutes
- Code 90834 = therapy given for 38 to 52 minutes
- Code 90837 = therapy given for 53 minutes or greater

Family Counseling 90846-HF (without patient present)/ 90847-HF (with patient present)/ 90849-HF (multiple family group)

A recipient focused intervention that may include the recipient, his/her collateral* and a qualified practitioner. This service is designed to maximize strengths and to reduce behavior problems and/or functional deficits stemming from the existence of a mental health and/or substance use disorder that interferes with the recipient's personal, familial, vocational, and/or community functioning. *Family counseling that involves the participation of a non-Medicaid eligible is for the direct benefit of the beneficiary. The service must actively involve the beneficiary in the sense of being tailored to the beneficiary's individual needs. There may be times when, based on clinical judgment, the beneficiary is not present during the delivery of the service, but remains the focus of the service.*

Billing Unit:	1 episode=minimum of 60 minutes (90846-HF/ 90847HF) 1 episode=minimum of 90 minutes (90849-HF)
Maximum Units:	1 episode per day, 104 per year
Billing Restrictions:	May not be billed in combination with Partial Hospitalization (H0035), Opioid Use Disorder Treatment (H0020), Outpatient Detoxification – Ambulatory Detoxification With Extended On-Site Monitoring (II-D) (H0013), Outpatient Detoxification – Ambulatory Detoxification Without Extended On-Site Monitoring (I-D) (H0014).

Additional Information:
Family therapy is defined as the treatment of family members as a family unit, rather than an individual patient. When family therapy without the patient present (90846) or family therapy with the patient present (90847) is provided, the session is billed as one service (one family unit), regardless of the number of individuals present at the session.

When a family consists of a Medicaid eligible adult and child(ren) and the therapy is not directed at one specific child, services may be directed to the adult for effective treatment of the family unit to address the adult's issues and impact on the family. If the adult is not eligible and the family therapy is directed to the adult and not the child, the service may not be billed using the

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child's recipient id number and the service must actively involve the beneficiary in the sense of being tailored to the beneficiary's individual needs.

If there is more than one eligible child and no child is exclusively identified as the primary recipient of treatment, then the oldest child's recipient id number must be used for billing purposes. When a specific child is identified as the primary patient of treatment, that child's recipient ID number must be used for billing purposes. A family may be biological, foster, adoptive or other family unit.

A family is not a group and providers may not submit a claim for each eligible person attending the same family therapy session.

All members of the family in attendance for the session will sign/mark the signature log or progress note to document their participation in the session (in addition to the therapist documenting their presence/participation).

Group Counseling (90853-HF)

The utilization of professional skills by a qualified practitioner to assist two or more unrelated recipients in a group setting in achieving specific objectives of treatment or care for a substance use disorder. Services are generally directed toward alleviating maladaptive functioning and behavioral, psychological, and/or emotional disturbances, and utilization of the shared experiences of the group's members to assist in restoration of each participant to a level of functioning capable of supporting and sustaining recovery. Group Counseling may consist of insight oriented, behavior modifying, supportive, or interactive psychotherapeutic service strategies.

Billing Unit:	1 episode=minimum of 90 minutes
Maximum Units:	1 episode per day, 104 per year
Billing Restrictions:	May not be billed in combination with Partial Hospitalization (H0035), Opioid Use Disorder Treatment (H0020), Outpatient Detoxification – Ambulatory Detoxification With Extended On-Site Monitoring (II-D) (H0013), Outpatient Detoxification – Ambulatory Detoxification Without Extended On-Site Monitoring (I-D) (H0014).

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Treatment Plan Review (H0032)
Review and/or revision of a recipient's individualized substance use disorder treatment plan by a qualified practitioner who is not routinely directly involved in providing services to the recipient. This review will evaluate the recipient's progress toward treatment objectives, the appropriateness of services being provided, and the need for a recipient's continued participation in treatment.

Billing Unit:	15 minutes
Maximum Units:	1 event with up to 2 units per quarter, 1 event per day, 8 per year (for DMH-MI providers) 1 event with up to 2 units per quarter, 1 event per day, 8 per year (for DMH-SASD providers)
Billing Restrictions:	None

Substance Use Disorders Update (H0031-HF)
A structured interview process that functions to evaluate a recipient's present level of functioning and/or presenting needs. The assessment is used to establish additional or modify existing diagnoses, establish new or additional goals, assess progress toward goals, and/or to determine the need for continued care, transfer, or discharge.

Billing Unit:	15 minutes
Maximum Units:	8 units per day, 56 units per year
Billing Restrictions:	May not be billed in combination with Intake Evaluation (90791)

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Psychoeducational Services (H2027-HF – Individual; H2027-HQ – Group)
Structured, topic specific educational services provided to assist the recipient and the families* of recipients in understanding the nature of the identified behavioral health substance use disorder, symptoms, management of the disorder, how to help the recipient be supported in the community and to identify strategies to support restoration of the recipient to his/her best possible level of functioning.

Billing Unit:	15 minutes
Maximum Units:	416 units per year *8 per day for services provided to an individual recipient's family* *8 per day for services provided to a group of recipients' families*
Billing Restrictions:	May not be billed in combination with Opioid Use Disorder Treatment (H0020), Outpatient Detoxification – Ambulatory Detoxification With Extended On-Site Monitoring (II-D) (H0013), Outpatient Detoxification – Ambulatory Detoxification Without Extended On-Site Monitoring (I-D) (H0014) and H0035-HF Partial Hospitalization.

Opioid Use Disorder Treatment (H0020: HF - Methadone, H0020: HF:AM – Buprenorphine, J2315: HF - Vivitrol)
The administration of medication, including the use of FDA approved medications for the use of opioid use disorders, to recipients who have a diagnosed opioid use disorder. Medication is administered to support the recipient's efforts to restore adequate functioning in major life areas that have been debilitated as a result of opioid addiction. This service includes medication administration and concurrent related medical and clinical services.

The program must be staffed as specified in current and subsequent revisions of:

(1) State regulations established for this service by the Alabama Department of Mental Health and published in the Alabama Administrative Code; and

(2) Federal regulations established for this service by the Substance Abuse and Mental Health Services Administration

Billing Unit:	One day
Maximum Units:	365 per year for H0020 (oral Methadone, Buprenorphine). 1 per month for J2315 (injectable Vivitrol)

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Additional Information:
H0020 may be span-billed by multiplying the appropriate number of units for the month by the daily rate. Benefits are limited to 365 units per year. Please utilize the AM modifier when billing for Buprenorphine. Utilization will be monitored through retrospective reviews.

Peer Support Services H0038 HF: HB (Adult) or HF: HB: HQ (Group)
Peer Support services provides structured, scheduled activities that promote socialization, recovery, self-advocacy, development of natural supports, and maintenance of community living skills, by Certified Recovery Support Specialists. Peer Support service actively engages and empowers an individual and his/her identified supports in leading and directing the design of the service plan and thereby ensures that the plan reflects the needs and preferences of the individual (and family when appropriate) with the goal of active participation in this process. Additionally, this service provides support and coaching interventions to individuals (and family when appropriate) to promote recovery, resiliency and healthy lifestyles and to reduce identifiable and increase healthy behaviors intended to prevent relapse and promote long-term recovery. Peer supports provide effective techniques that focus on the individual's self-management and decision making about healthy choices, which ultimately extend the members' lifespan.

Billing Unit:	15 minutes
Maximum Units:	Limited to 20 units per day (individual) and 8 units per day (group). 2,080 units per year for group services and 2,080 units per year for individual services.
Billing Restrictions:	None

M.4. Requirements for Recipient Intake, Treatment Planning, and Service Documentation

An intake evaluation must be performed for each recipient considered for initial entry into organized programs or course of covered services. Individuals who are transferred between programs within an agency do not require a new intake at the time of transfer.

To determine a recipient's, need for MAT services, providers must perform an intake evaluation (utilizing the Standardized Substance Abuse Psychosocial Assessment) based on assessment of the following information:

- Family history
- Educational history
- Relevant medical background
- Employment/vocational history
- Psychological/psychiatric treatment history
- Military service history
- Legal history

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- Alcohol/drug use history
- Mental status examination
- A description of the significant problems that the recipient is experiencing

A written treatment plan (service plan, individualized family service plan, plan of care, etc. must be completed by the fifth face-to-face outpatient services, within ten working days after admission in all day programs or residential program, or within other time limits that may be specified under programs specific requirements. For SA services, the patient, counselor, and licensed staff as noted above must all approve the treatment plan prior to the provision of SA treatment services.

- Identification of the clinical issues that will be the focus of treatment
- Specific services necessary to meet the recipient's needs
- Referrals as appropriate for needed services not provided directly by the agency
- Identification of expected outcomes toward which the recipient and therapist will be working to impact upon the specific clinical issues

Unless clinically contraindicated, the recipient will sign/mark the treatment plan to document the consumer's/recipient's participation in developing and/or revising the plan. If the recipient is under the age of 14 or adjudicated incompetent, the parent/foster parent/legal guardian must sign the treatment plan.

Additional Information:
Services may be provided and billed between the initial intake service and the development of the treatment plan if provided within the allowed timeframe for treatment plan development. Once the treatment plan is developed, service types must be specified in the treatment plan in order to be paid by Medicaid, with the exception of intake evaluation, substance use disorders assessment update, and treatment plan review. Changes in the treatment plan must be approved as described above.

After completion of the initial treatment plan, staff must review the recipient's treatment plan once every three months to determine the recipient's progress toward treatment objectives, the appropriateness of the services furnished, and the need for continued treatment. Providers must document this review in the recipient's clinical record by noting on the treatment plan that it has been reviewed and updated or continued without change. Staff, as specified above, must perform this review. The treatment plan review must be completed and signed by the appropriate staff in the required time frame in order to continue billing for services.

Treatment plan reviews are not covered in cases where only an intake or diagnostic assessment is provided with no further treatment. One treatment plan review is covered following a three-month interval of no services delivered. Any subsequent reviews with no intervening treatment are disallowed.

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M.5. Documentation

Documentation in the recipient's record for each session, service, or activity for which Medicaid reimbursement is requested must comply with any applicable certification or licensure standards and must include the following, at a minimum:

- The identification of the specific services rendered
- The date and the amount of time that the services were rendered (to include the time started and the time ended)
- The signature of the staff person who rendered the services
- The identification of the setting in which the services were rendered
- A written assessment of the recipient's progress, or lack thereof, related to each of the identified clinical issues discussed

All entries must be legible and complete and must be signed and dated by the person (identified by name and discipline) who is responsible for ordering, providing, or evaluating the service furnished. The author of each entry must be identified and must sign his or her entry.
Documentation of Medicaid recipients' signatures may be entered on a sign-in log, service receipt, or any other record that can be used to indicate the recipient's signature and the date of service.

Treatment plan review and any non-face-to-face services that can be provided by telephone do not require recipient signatures.

When clinical records are audited, Medicaid will apply the list of required documentation to justify payment. Documentation failing to meet the minimum standards noted above will result in recoupment of payments.

Documentation should not be repetitive (examples include, but are not limited to the following scenarios):

- Progress Notes that look the same for other recipients.
- Progress notes that state the same words day after day with no evidence of progression, maintenance or regression.
- Treatment Plans that look the same for other recipients.
- Treatment Plans with goals and interventions that stay the same and have no progression.
- Progress Notes should not be preprinted or predated with the exception that a group therapy note may have a general section that identifies the participants (i.e. the number of participants, etc.), the topic, and a general description of the session which is copied for each participant. However, each participant must also have individualized documentation relative to his/her specific interaction in the group and how it relates to their treatment plan. The progress note should match the goals on the plan and the plan should match the needs of the recipient. The interventions should be appropriate to meet the goals. There should be clear continuity between the documentation.
- Progress Notes must provide enough detail and explanation to justify the amount of billing.

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Treatment Plan

- The Treatment Plan should not be signed or dated prior to the plan meeting date.

Authentication

- Authors must always compose and sign their own entries (whether handwritten or electronic). An author should never create an entry or sign an entry for someone else or have someone else formulate or sign an entry for them. If utilizing a computer entry system, the program must contain an attestation signature line and time/date entry stamp.
    - If utilizing a computer entry system, there must be a written policy for documentation method in case of computer failure/power outage.

Corrections

- Corrections must be made legally and properly by drawing a line through the entry and making sure that the inaccurate information is still legible. Write “error” by the incorrect entry and initial. Do not obliterate or otherwise alter the original entry by blacking out with marker, using whiteout, or writing over an entry. White Out, Liquid Paper, or any form of correctional fluid or correctional tape is not acceptable on **any** records whether being used as a corrective measure or to individualize an original template or for any other reason.

NOTE:

Please refer to Chapter 105 Sections 105.3 – 105.6 for all other information related to billing of claims.

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